

Chapter V

The Information Asymmetry Condition

The Right to Know Before Consent

The Upstream Condition

Chapter IV closed on a forward reference. The chapter that preceded examined whether the consultation undertaken in connection with Bill 12 met the standard CRPD Article 4(3) requires. The documented record established that it did not. That chapter closed by naming the upstream condition without which the consultation standard cannot operate: the right of the affected population to know, in advance, what they are being asked to consent to.

This chapter examines that condition. The claim it advances is narrow and structural. Consent presupposes information. A population that is not in a position to know the operational rules of the program being designed for them, the criteria against which their continued eligibility will be assessed, or the standards by which the documentation they are required to produce will be evaluated, cannot meaningfully consent to the program. The consultation standard CRPD Article 4(3) requires is operationally hollow where the upstream information condition is not met. The hollow standard is the standard the Government of Alberta operated under in 2025. The chapter that follows documents how.

The Standards That Apply

Two articles of the Convention on the Rights of Persons with Disabilities are directly engaged. Article 9, the accessibility provision, requires State Parties to take appropriate measures to ensure that persons with disabilities have access, on an equal basis with others, to information and communications, including information and communications technologies and systems. Article 21, the freedom of expression and access to information provision, requires State Parties to ensure that information intended for the general public is provided in accessible formats and technologies appropriate to different kinds of disabilities, in a timely manner and without additional cost.¹

The domestic framework adds further specificity. The federal *Accessible Canada Act*, SC 2019, c 10, enacted as the domestic operationalization of Canada's CRPD obligations in the federal sphere, establishes the statutory goal of a Canada without barriers by January 1, 2040. The Act binds federally regulated organizations and federally delivered programs. The federal components implicated in the disability-support architecture — the Canada Revenue Agency, Service Canada, the Disability Tax Credit, and the Canada Disability Benefit — are within scope.²

At the provincial level, the *Alberta Human Rights Act* prohibits discrimination on the basis of physical and mental disability in the provision of government services. Section 4 establishes an affirmative duty of accommodation triggered when the service provider is on notice that a barrier exists — not when the affected individual files a formal request. At the constitutional level, Section 7 of the Canadian Charter of Rights and Freedoms protects against deprivation of life, liberty, or security on the basis of evidence the affected person cannot review, criteria they cannot evaluate, or processes in which they cannot meaningfully participate. The Supreme Court of Canada in *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624, established that Section 15 of the Charter requires substantive equality — not merely identical text on paper, but the actual capacity of protected groups to exercise their rights on equal terms.³

Plain-language standards operationalize what accessibility means for written communications. The federal Treasury Board Secretariat recommends a Grade 8 reading level or below for general public communications. The disability-advocacy community recommends Grade 6 or below for communications directed at populations with cognitive or intellectual disabilities. The federal Plain Language Action and Information Network defines plain language as writing that the intended audience can read, understand, and act on the first time they read it. Statistics Canada’s Programme for the International Assessment of Adult Competencies (PIAAC) data places approximately forty-nine percent of Canadian adults below the literacy threshold required for everyday government and health communications.⁴

The Reading-Level Gap

Independent readability analysis of Government of Alberta AISH program documentation, ADAP transition information, and federal Canada Disability Benefit application materials places the typical reading level between Grade 12 and Grade 16 — that is, university-undergraduate to graduate-level English. A document at this reading level is comprehensible to approximately twelve percent of Canadian adults at first-read full comprehension.⁵

The reading level matters because of who is in the AISH caseload. The Government of Alberta’s own caseload data as of September 2025 identifies primary medical conditions across the 79,290 recipients as follows: 22,419 (28.3 percent) with cognitive disorders as their primary diagnosis, including autism, intellectual disability, acquired brain injury, and severe dementia; 23,940 (30.2 percent) with mental illness as their primary diagnosis, including major depressive disorder, severe anxiety, schizophrenia, and post-traumatic stress disorder; approximately 24,800 (31.3 percent) with physical disability, including mobility, sensory, and chronic-illness conditions; and approximately 8,131 (10.2 percent) with other or multiple primary diagnoses. Combined, more than 58 percent of the AISH caseload — 46,359 people — have a primary diagnosis of cognitive disorder or mental illness.⁶

The defining clinical features of these conditions include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, difficulty navigating institutional systems without external support, and difficulty maintaining sustained attention across multi-page documents. These are not aspersions. They are the diagnostic criteria. They are the basis on which the Government of Alberta approved each recipient as

eligible for AISH in the first place.

The same Government of Alberta then designed a compliance system requiring those same recipients to read forms written at Grade 12 to 16 reading level, to complete multi-step paperwork chains across multiple agencies, to find a family doctor in a province where approximately 650,000 people have no family physician, to pay physician fees out of pocket on an income below the poverty line, and to navigate phone trees, online portals, and appeal processes without integrated accommodation. The defining clinical features of the population the program was designed to serve are the defining features the program then requires the population to override in order to remain on it.⁷

What Was Not Published Before the Transition

The information that the Government of Alberta did not publish, in any form that a prospective ADAP recipient could review before the consultation closed in September 2025, is the category of information most directly relevant to the consent question Chapter IV examined. The list is documented in the campaign's *AISH Evidentiary Report Series* and the *Compound Failure Brief*; the items below are reproduced with the campaign's primary-source verification.⁸

The basis for the reversal of prior findings. Every AISH recipient was assessed and approved by the Government of Alberta's formal adjudication process. The ADAP transition reverses those findings for all 79,290 recipients simultaneously. The Government has not published, in any form, the medical, administrative, or legal basis on which the prior findings are treated as insufficient to maintain eligibility without reapplication. The reversal is operative; the justification is undocumented.

The criteria distinguishing AISH from ADAP eligibility. Minister Jason Nixon confirmed in correspondence dated April 3, 2026 that the employment income exemption calculations under the post-transition framework would be set in a Ministerial Order "later this spring." The transition is effective July 1, 2026. The operational rules governing the income reduction structure that will apply to 79,290 people were not published when those people were consulted in the engagement window that closed in September 2025. They were not published when Bill 12 was passed in December 2025. They were not published as the campaign's most recent documentation of the matter went to publication in May 2026.⁹

The Medical Review Panel adjudication standards. The Panel that replaces the Citizens Appeal Panel as the final decision-maker on AISH eligibility under the post-transition framework will apply criteria the Government has not published. The specific criteria the medical adjudicator applies to determine "substantial limitation of ability to earn a livelihood" have not been disclosed. The evidentiary thresholds for "permanence" of a condition have not been disclosed. The assessment standards for fluctuating conditions — which do not present consistently across any single assessment period and which characterize many of the mental health and chronic-pain conditions in the caseload — have not been disclosed. The criteria for determining "likely improvement," a determination that requires the adjudicator to make a prognosis the applicant's own physicians may not have made, have not been disclosed.¹⁰

The structural “framing-amenable” criterion. The campaign’s *Compound Failure Brief* identifies an operative criterion that the Medical Review Panel will, in functional effect, apply: not whether a recipient’s disability is real or severe (the existing file already establishes that, and the Government has accepted the equivalent established record for every recipient in an exempted category) but whether a recipient can be plausibly framed as a candidate for employment. The exemption categories themselves identify the criterion: recipients over sixty are exempted because their employment trajectory is no longer considered relevant; PDD recipients are exempted because their developmental disability is considered conclusive of unemployability; continuing-care residents are exempted because their living situation is conclusive; palliative recipients are exempted because their prognosis is conclusive. The transitioning cohort is the cohort the Government considers framing-amenable. The criterion is not in any published document. Its operation is documented in the structure of the exemptions.¹¹

The recipient’s position. The applicant whose file is being newly evaluated under this unpublished criterion has no opportunity to review what is being applied to them, no opportunity to respond to it, and no independent body to which an erroneous application can be appealed. The criterion is being applied by appointees of the ministry whose budget benefits from each ADAP placement, in the absence of the appeal panel that historically reviewed decisions of this kind. The Government’s existing record on the recipient — the record establishing that the same Government previously found this person unable to sustain a livelihood — is held in the same ministry’s filing cabinet as the new determination is being made.

The Compounding Effect Across Programs

The information-asymmetry pattern is not unique to AISH and ADAP. It compounds across the federal-provincial disability-support chain. The federal Canada Disability Benefit requires prior approval for the Disability Tax Credit. The DTC requires a physician to complete the T2201 form. Physician fees for T2201 completion are unregulated and range from approximately fifty to three hundred dollars or more across the province. The Government of Canada’s 2024 Disability Advisory Committee report documents a twenty-four percent completion rate for online DTC applications — equivalently, a seventy-six percent abandonment rate. The Disability Advisory Committee identifies administrative complexity as a documented access barrier rather than a measure of ineligibility.¹²

The same population that must complete the DTC to qualify for the CDB is the population whose CDB Alberta then claws back dollar-for-dollar from the provincial AISH or ADAP cheque. The documentation chain is documented in the campaign’s *Access Barrier Report* and traced through timestamped correspondence in a primary-source case file spanning 2019 to 2026. The chain’s practical effect is that a recipient who succeeds in completing all of the federal access requirements receives zero net financial benefit, and a recipient who fails in completing them sees their AISH cheque reduced by two hundred dollars per month regardless.¹³

The same pattern operates in the child and developmental-disability programs. The Family Support for Children with Disabilities (FSCD) and Persons with Developmental Disabilities (PDD) program forms inherit the same readability and accessibility issues with one structural addition: the parent or guardian

completing the form is often themselves managing the disability of the child or family member requiring the support. The administrative complexity does not adjust for caregiver load. The bureaucratic shorthand does not soften when the household is in crisis. Families drop out of programs they were eligible for, or never apply at all.¹⁴

The Structural Contradiction

The integrated effect of the documented pattern is a structural contradiction that can be stated precisely. The Government of Alberta determined, through its own formal adjudication process, that 79,290 people have severe and permanent disabilities preventing them from earning a livelihood. The defining clinical features of the largest groups in that population — cognitive disorders, mental illness — include difficulty with sustained executive function, difficulty processing complex written communication, difficulty initiating and completing multi-step administrative tasks, and difficulty navigating institutional systems without external support. The same Government then designed the compliance, consultation, and reapplication architecture under which the same population is required to read forms at Grade 12 to 16 reading level, navigate multi-step bureaucratic processes across multiple agencies, meet deadlines without flexibility for medical flares or hospitalizations, and respond to consultation invitations whose subject matter is the criteria that will subsequently be applied to determine their continued eligibility — criteria the Government has not published.¹⁵

The contradiction is not incidental. It is the operating structure of the program. The recipient population was assessed and approved into AISH because of the limitations that make the compliance system inaccessible. The same limitations are then operationally treated, by the program's communications and processes, as if they were not present. A population whose qualifying conditions include difficulty with multi-step administrative tasks is required to perform multi-step administrative tasks in order to keep the support that recognized the difficulty in the first place.

The community response to the documented pattern is itself a data point. The Alberta AISH recipient community group tripled in size over the period from spring 2025 to spring 2026, from approximately two thousand members to more than six thousand, according to the group's administrator. Community groups of this kind do not triple in size when recipients feel supported and informed. They triple when recipients feel abandoned and afraid, and when they are actively seeking information and community that the formal system has ceased to provide. The growth of informal recipient-led networks during the same period the formal supports were being removed is not coincidence. It is the predictable consequence of the removal.¹⁶

The Constructive Knowledge Doctrine

A principle in administrative law and in human rights jurisprudence operates across the situation this chapter documents. Where an institution knows, or where in the circumstances it ought to know, that a standard it is applying cannot be met by the population to whom it is applied, the institution's continued application of the standard is not procedurally innocent. The constructive-knowledge doctrine attaches the institution's knowledge to its conduct regardless of whether each individual decision-maker subjectively intends the foreseeable result.¹⁷

The Government of Alberta is on documented constructive notice of every element examined in this chapter. The 79,290 caseload figure and the breakdown by primary medical condition are from its own open data. The Grade 12 to 16 reading level of its forms is a function of its own document drafting practice. The non-publication of the Medical Review Panel adjudication standards is its own administrative choice. The Ministerial Order setting the employment income exemption calculation is within its own authority to issue or withhold. The 24 percent DTC completion rate is documented in federal reports the Government has the operational capacity to consult. The reading-level standards in federal Treasury Board guidance, in disability-advocacy practice, and in the Convention itself are public and accessible.

The Government has been formally notified of the pattern documented here through Inclusion Alberta's public analyses, through the campaign's evidentiary reports published and distributed to government addressees, through municipal council motions, through the November 26, 2025 Inclusion Alberta media release, through the MS Canada statement of November 28, 2025, through Alberta Medical Association publications including the November/December 2025 issue of *Alberta Doctors' Digest*, and through direct ministerial correspondence from named individual recipients. The constructive knowledge condition is met as a matter of documented record.¹⁸

Where the constructive knowledge condition is met, the continued operation of the inaccessible standard is not an administrative failure subject to incremental reform. It is the operating design of the program. The information-asymmetry condition is, on the documented record, sustained because it produces the outcomes the surrounding fiscal architecture serves. The recipient who cannot complete the reapplication does not remain on AISH. The applicant who cannot navigate the DTC chain does not access the CDB. The family that cannot parse the FSCD documentation does not receive the support. The caseload reduces by attrition. The fiscal saving is the saving the policy framework was structured to produce.

What This Chapter Establishes

Four observations follow.

First, the CRPD Article 4(3) consultation standard examined in Chapter IV cannot meaningfully operate where the upstream information condition is not met. A population that has not been informed of the operational rules of the program being designed for them, of the criteria against which their continued eligibility will be assessed, or of the standards by which their documentation will be evaluated, cannot meaningfully consent to the program through any consultation mechanism the consulting authority designs.

Second, the documented information condition in Alberta as of the consultation window in September 2025 and as of the transition window in July 2026 fails the standards CRPD Articles 9 and 21 establish, the standards the federal *Accessible Canada Act* articulates, and the standards the *Alberta Human Rights Act* Section 4 duty of accommodation triggers. The failure is not incidental. It is documented in the reading-level of the forms, in the non-publication of the adjudication standards, in the deferral of the Ministerial Order, and in the structural contradiction between the qualifying conditions and the navigational demands.

Third, the structural contradiction is precise. The population the program was designed to serve was assessed and approved because of the limitations that make the program's communications and processes inaccessible. The same limitations are then operationally treated as if they were not present. The diagnostic criteria on which approval was granted are the diagnostic criteria the maintenance system then requires the recipient to override.

Fourth, the constructive knowledge doctrine is engaged on the documented record. The Government of Alberta is on notice, through its own open data, through federal accessibility jurisprudence, through formal correspondence from civil society organizations, and through municipal advocacy, of every element of the pattern this chapter documents. The continued operation of the inaccessible architecture, in the face of the documented pattern, is not an administrative failure capable of incremental reform without a change in the institutional position. It is the operating structure that produces the fiscal outcomes the surrounding policy framework was designed to produce.

The chapter that follows examines what is being built in the institutional space the consultation standard would have constrained — the architecture currently under construction in Alberta and in jurisdictions adjacent to it. The information condition documented in this chapter is the condition under which that construction has been authorized to proceed.

Notes — Chapter V

1. *Convention on the Rights of Persons with Disabilities*, Articles 9 (Accessibility) and 21 (Freedom of Expression and Access to Information); Canada ratified March 11, 2010. The full text of both articles is available at the UN Department of Economic and Social Affairs, Division for Inclusive Social Development, <https://www.un.org/development/desa/disabilities/>.
2. *Accessible Canada Act*, SC 2019, c 10; Bill C-81, *An Act to Ensure a Barrier-Free Canada*, tabled June 20, 2018 by the Honourable Carla Qualtrough, then Minister of Public Services and Procurement and Accessibility; Royal Assent June 21, 2019; in force July 11, 2019. Statutory objective: realization of a Canada without barriers by January 1, 2040.
3. *Alberta Human Rights Act*, RSA 2000, c A-25.5, section 4; *Canadian Charter of Rights and Freedoms*, sections 7 and 15, Part I of the *Constitution Act, 1982*; *Eldridge v. British Columbia (Attorney General)*, [1997] 3 SCR 624 (substantive equality requires accommodation rather than merely identical text on paper).
4. Government of Canada, Treasury Board Secretariat, Communications Policy of the Government of Canada and associated plain-language guidance; Plain Language Action and Information Network (PLAIN), plainlanguage.gov; Statistics Canada, Programme for the International Assessment of Adult Competencies (PIAAC), Adult Literacy and Life Skills Survey data.
5. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5 (Bureaucratic Legalese as Gatekeeping). Independent readability analysis using standard tools (Flesch-Kincaid Grade Level) of Government of Alberta AISH and ADAP documentation and federal CDB application materials.
6. Government of Alberta, AISH Caseload Open Data, September 2025, open.alberta.ca; The Alberta Disability System Breakdown, *What Disability Actually Means*, May 2026 (plain-language guide documenting the caseload breakdown by primary medical condition).
7. Government of Alberta, family physician availability data; The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 1 (The Structural Contradiction).
8. The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, Section 6 (The Questions That Require Answers Before July 1, 2026); *Compound Failure Brief*, April 2026, Pillar III(d) (the Removal of Appeal Rights and the Medical Review Panel architecture).
9. Minister Jason Nixon, Minister of Assisted Living and Social Services, letter to Genevieve Bissonette, April 3, 2026 (confirming the Ministerial Order on employment income calculations would be issued “later this spring”); The Alberta Disability System Breakdown, *AISH Evidentiary Report Series*, April 2026, on the deferred Ministerial Order.
10. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), documenting the specific categories of adjudication criteria that have not been published in any form a prospective applicant could review before the post-transition Medical Review Panel adjudication.
11. The Alberta Disability System Breakdown, *Compound Failure Brief*, April 2026, Pillar III(d), analysis of the exemption-category structure and the operative “framing-amenable” criterion derived from it.
12. Government of Canada Disability Advisory Committee, 2024 report; documented 24 percent completion rate for online DTC applications and 76 percent abandonment rate. DTC physician fee structure documented in The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, with primary-source receipts and correspondence.
13. The Alberta Disability System Breakdown, *Access Barrier Report*, April 2026, documenting the federal-provincial DTC/CDB/AISH chain through timestamped correspondence in a primary-source case file spanning 2019 to 2026.
14. The Alberta Disability System Breakdown, *The Access Gap: Penalized for Non-Ability*, May 2026, Section 5.3 (The FSCD/PDD Form Comparison). The pattern of caregiver-mediated administrative complexity inheriting the same readability and accessibility issues as the recipient-mediated AISH/ADAP architecture.

15. See note 7 and accompanying text; The Alberta Disability System Breakdown, *Information Asymmetry Plain Language Companion*, April 2026, naming the structural contradiction at the policy-design level.
16. Community group growth documented in the campaign’s observations of the Alberta AISH recipient community group — the same kind of network that emerged in disability communities historically when formal support structures failed; see The Alberta Disability System Breakdown, *Indigenous Disability Alberta Collection*, May 2026, for the parallel pattern in Indigenous disability communities during the same period.
17. The constructive-knowledge doctrine operates across multiple branches of Canadian and international law, including administrative-law jurisprudence on procedural fairness, human-rights jurisprudence on the duty to accommodate (see, e.g., *British Columbia (Public Service Employee Relations Commission) v. BCGSEU*, [1999] 3 SCR 3 (the *Meiorin* standard)), and tort jurisprudence on knowing or willful continuation of harmful conduct. The doctrine’s application to the disability-policy context is developed in The Alberta Disability System Breakdown, *Charter and CRPD Analysis*, April 2026.
18. Inclusion Alberta, “Bill 12 erodes rights and deepens poverty for Albertans with disabilities,” media release, November 26, 2025; MS Canada, November 28, 2025; Alberta Doctors’ Digest, “Pushing back against changes that make life harder for disabled Albertans,” November/December 2025; The Alberta Disability System Breakdown distribution record covering Tier 1 (provincial, federal, oversight, municipal), Tier 2 (coalition), and Tier 3 (media) recipients, 2025–2026.